

جوستاف روسي انترناسيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إِسْمُ الْمَرِيضِ رِبَاعِيًّا : كَفَايَهُ سَيِّدُ مُحَمَّدٍ سَلَامُ

المحافظة : القاهرة

الحي / المدينة : البساتين

العنوان : ...

رقم البطاقة : 26208270101281 رقم التليفون : 01103012802

تاريخ تسجيل الإستمارة : 28-09-2026

العمر(تاريخ الميلاد) : 27-08-1962 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي (-----)
المقر بما فيه التوقيع: (-----)

التوقيع: (_____)

المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام اشعة مقطعية صدر مع صبغة [10851] **قسم العلاج :** الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة:

الوظيفة:

الوظيفة:

التوقع:

التوقع:

التوقيع:

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

قسم :

نفقات :

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: كفايه سيد محمد سلام National ID: 26208270101281	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT of the chest.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

كفالة
سيد محمد سلام
٢٤ ش عبد سليمان - عبد الخالق الطحاوي
البيستين - القاهرة

١٩٩٢/٠٨/٢٧ ٢٦٢٠٨٢٧٠١٠١٢٨١

KQ8943745

٢٠٢٤/٠٧ ٢٦٢٠٨٢٧٠١٠١٢٨١

ربة منزل

أنثى
فتحي امام مطاوع سليمان
البطاقة مصرية حتى ٢٠٢١/٠٧/٢٢

١

Patient's Name: kafaia sayed mohamed salam
Date: 2026-04-05
Referred by: Prof.Dr. et cap iv
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported left breast carcinoma , compared to last study dated on 13/12/2025.

(I) Chest:

- Most of Left breast is out of field of examination , however breast edema is noted .
- Mixed course regarding bilateral enlarged axillary lymph nodes , the largest on right side measures 3 x 2.3 cm compared to 4 x 2.4 cm and on left side 2.6 x 1.2 cm compared to 3.2 x 2.7 cm .
- Regressive course regarding right cardiophrenic angle L N 13 x 7 mm .
- Few scattered pulmonary nodules , the largest at right upper lobe 6mm at middle lobe.
- Middle lobe thick atelectatic band .
- Clear both lung fields .no sizable pulmonary masses or consolidation patches
- few enlarged mediastinal lymph node , the largest seen at AP window measures 13 x 8 mm

Regressive
22

Please
confirm
size

& comment

Name: كفايه سيد محمد سلام		
MR. : 1810		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 64-year-old individual who is postmenopausal. The clinical history documents a prior left-breast cancer diagnosed at age 63, which was estrogen-receptor negative, progesterone-receptor negative, and HER2-positive. The disease progressed with metastases to the right axilla, liver, lung, and lymph nodes. Prior therapy included 33 weeks of taxol and trastuzumab, followed by four cycles of taxol plus carboplatin, then trastuzumab, docetaxel, capecitabine, and lapatinib from May 2025 to September 2025, and subsequently docetaxel, gemcitabine, and carboplatin in December 2025. The patient continued with ten cycles of gemcitabine and carboplatin. On examination, a fungating left-breast mass measuring 6 × 7 cm was noted on 20 July 2026; there was no hepatic focal lesion (previous CT showed bilateral hepatic focal lesions, the largest 3.8 × 2.4 cm). Bilateral axillary lymph nodes have decreased in size, and pulmonary nodules are now only 8 mm, suggesting response to systemic therapy. Imaging confirms the left-breast fungating mass with local progression, while metastatic sites (bilateral hepatic lesions, axillary nodes, and pulmonary nodules) demonstrate stability or reduction. No new distant disease is identified on the latest work-up. The MDT recommends initiation of ado-trastuzumab emtansine (Kadcyla) provided by a non-governmental organization, with consideration of palliative radiation to the locally progressive fungating breast lesion, while maintaining the current gemcitabine and carboplatin regimen given the systemic stable disease/partial response.		
تحريرا في ,		
	الوقت :	التاريخ : 23-09-2026
		الطبيب المعالج :
يعتمد ,		

Patient's Name: kefaia sayed mohamed salam
Date: 2026-04-05
Referred by: Prof.Dr. ct cap iv
Hospital: Goustave Roussey International Egypt

- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous CT density , with bilobar hepatic focal lesions the largest at segment V shows regressive course measuring 3.8 x 3.6 cm compared to 5.6 x 5.4 cm .
- Large gall bladder stone measures 2.3 x 1.7 cm .
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- Normal CT appearance of the spleen .
- Regular filling of the urinary bladder showing no stones, masses .
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- No worrisome lytic or sclerotic bony lesions .
- No vertebral structural collapse .

OPINION:

Left breast carcinoma.

- Mixed course regarding bilateral enlarged axillary lymph nodes
- Regressive course regarding rght cardiophrenic angle L N
- Regressive course regarding size of bilobar hepatic focal lesions

Dr.Reham Khedr, MD

Patient Name	كفايه سيد محمد	Age	63
Sex	F	Special habits	
Family History		Comorbidity	
Surgical History		Telephone	0102- 55506 0110- 38103
Menopausal Status	Postmenopausal	Marital Status	
ID		Medical ID	1685/ 4
Date	16/9/2025	Reason of MDT	

Medical Report

Pathology: Bx-> Left breast IDC GIII
ER -ve, PR -ve , Her2: +3
ECOG:1

T4b N3 M1 (Liver, LN, Lung)

S.M:

Metsworkup: (5/2024) Pet Ct : bilateral axillary lt mammary mediastin LNS, bilateral pulmonary nodules, HFL.

Recommendations: Taxol, Carboplatin, Herceptin 33wks +4 Herceptin only

3/2025: PD upon CT.

Xeloda and Tykerb

6/2025: Rec 4 cycles -> delayed

6/2025: Xeloda

9/2025CT: PD of bilateral Pulmonary nodules ,Mild progression in right cardiophemic LNs, Liver mildly enlarged showing PD regarding the size of the few previously seen bilobar hypodense focal lesions largest measuring 6.1x4.1cm at segment V

9/2025- Xeloda 4x3x14, Tykerb 5x1x30, Supportive ttt

FOR MDT

MDT DECISION

FOR GEMZAR/CARBOPLATIN
CONSIDER TRASTZUMAB FROM NGO

Name	د/ خالد عبدالعزيز	د/ هاشم عبدالرحمن	د/ ابو بكر د
Signature			

Patient Name: Kefayah Sayed Mohamed Sallam	Date : 02/09/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 63 Year(s)
Medical ID: 1685/2024	Pat. ID: 53573

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

***Reported case of left breast cancer**

***As Compared to previous study dated 5-3-2025 the current study showed:
Degraded image quality by motion artefact.**

I Chest:

- Heterogenous edematous left breast parenchyma with overlying skin thickening for sonomammography.
- **Progressive course regarding the size of the previously noted** bilateral axillary lymph nodes, the largest measuring 3.7 x 2.8 cm on the left side and 4.5 x 2.1 cm on the right side in today's study.
- **Progressive course regarding the number and size** of the previously seen scattered bilateral pulmonary nodules, largest is seen at right lower lung lobe measures about 0.9 cm.
- Mild progressive course regarding the previously seen right cardiophrenic lymph node measuring 19 mm in today's study.
- **Stationary course regarding the size of the previously noted** prominent mediastinal lymph nodes.
- No pulmonary areas of ground glass attenuation, consolidation or cavitary lesions.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross abnormalities.

II Abdomen & pelvis:

- The liver is mildly enlarged showing **progressive course** regarding the size of the few previously seen bilobar hypodense focal lesions largest measuring 6.1 x 4.1 cm at segment V in today's study.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Still noted calcular gall bladder.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Under filled urinary bladder.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- No sizable worrisome osseous lesions.

Opinion:

- Overall progressive course since last study.

Dr. Sherif Wagdy

Dr. Sherif Wagdy



Patient Name: Kefayah Sayed Mohamed Sallam	Date : 05/03/2025
Exam : C.T CHEST	Age : 62 Year(s)
Medical ID: 1685/2024	Pat. ID: 53573

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

****Reported case of left breast cancer**

****As Compared to previous study dated 27-11-2024.**

I Chest:

- **Progressive course regarding the size of the previously noted:**
 - Prominent left axillary level I and II lymph nodes, the largest at level I measures 3.3 cm as compared to 1.7 cm priorly.
 - Enlarged right axillary level I lymph nodes, the largest measures 3.3X1.7 comared to 2x1 cm priorly.
- **Stationary course regarding the size of the previously noted:**
 - Prominent mediastinal and right cardiophrenic lymph nodes.
 - ?Prominent left internal mamamry lymph node measures about 0.7 cm
- **Rather stationary course regarding the number** and size of the few scattered bilateral pulmonary nodules, largest is still seen at left upper lung lobe still measures about 0.7x0.6 cm.
- No pulmonary areas of ground glass attenuation, consolidation or cavitary lesions.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross abnormalities.

II Abdomen & pelvis:

- The liver is mildly enlarged showing **Rather stationary** segment VI and segment III hypodense focal lesions, laregst



Patient Name: Kefayah Sayed Mohamed Sallam	Date : 05/03/2025
Exam : C.T CHEST	Age : 62 Year(s)
Medical ID: 1685/2024	Pat. ID: 53573

at segment VI measures about 2.8 x 3.2 cm, on non contrast basis.

- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Still noted calcular gall bladder.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Under filled urinary bladder.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- No sizable worrisome osseous lesions.

Opinion:

- **Progressive course regarding the size of the previously noted:**
 - **Prominent left axillary lymph nodes.**
 - **Enlarged right axillary lymph nodes.**
- **Rather stationary course of the pulmonary nodules as scribed**
- **Rather stationary rest of the study.**

Dr.Fatima Ahmed

Dr. Walid Mohamed, M.D.



Patient Name: Kefayah Sayed Mohamed Sallam	Date : 09/05/2024
Exam : MAMMOGRAPHY BOTH	Age : 61 Year(s)
Medical ID: 26208270101281	

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Left breast mass

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast marked edema pattern (increased skin thickening and coarse trabeculation)
- Right benign macrocalcifications
- No right suspicious micro calcifications or architectural distortion detected.

Complementary ultrasound :

- Left breast marked increased skin thickening with intra-parenchymal edema, no underlying definite mass lesions seen. Associated with skin inflammatory changes.
- No right cystic or solid mass lesions.
- Normal right skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Multiple left (level I and II) axillary pathological lymph nodes are seen.
- Pathological left supraclavicular lymph nodes.
- Pathological right axillary (level I) lymph nodes are seen.
- No pathological right infra or supraclavicular lymph nodes.

Opinion:

- Left breast locally advanced malignant looking lesion with pathological axillary and supraclavicular lymph nodes for histopathological correlation (BIRADS 5)
- Right axillary pathological lymph nodes likely metastatic for biopsy.
- No suspicious lesions in the right breast (BIRADS 2)

Yours truly ,

DR/Asmaa Zeinhom, Msc

Patient Name : الأستاذة / كفاية سيد محمد	Request No. : 130080
Patient No. : 26208270101281	Request Date : 14/05/2024
Path No. : 1556/24	Verification Date : 11/06/2024
Sex : Female	Financial Category : مبادره - اورام دار السيد
Age : 62 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs. Marwa Hegazy

Pathology Report

Clinical Data: Left breast swelling with malignant left axillary lymph node.
Nature of Specimen: Tru-cut biopsy.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- Her2 clone (4B5), and SISH.

Results:

- * IPT (Positive control showed appropriate reactivity):
 - Her2: Moderate incomplete membrane reaction in >10% of tumor cells (score 2+).
 - SISH: Amplified with clusters.
- (Average Her2 copy No.: (6), Her2/CEP17 ratio: (4))

Conclusion:

Left breast swelling with malignant left axillary lymph node, Tru-cut biopsy:
Invasive duct carcinoma, Grade III.

IPT:

- Her2: Equivocal (score 2+).
- SISH: Amplified with clusters.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and

Verified by :

Dr. Marwa mahmoud

Printing Date : 11/06/2024 14:08:39

Page 1 of 2

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name الأستاذة / كفاية سيد محمد
 Patient No. 26208270101281
 Path No. 1556/24
 Sex Female
 Age 62 Y
 Referring Doc.

Request No. 13008075
 Request Date 14/05/2024
 Verification Date 27/05/2024
 Financial Category اورام دار السلام - مبادره
 Department Out Patient

Registered by Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

Quantification: "1+ or 2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Prof. Dr. Lobna Omar.

Dr. Menna Nabil.

Verified by :

28/05/2024 09:26:04

Page 2 of 2

Printed By : Mrs.Mayada Hegazy

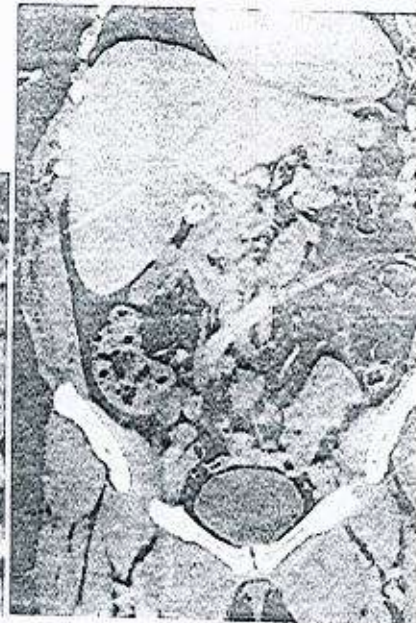
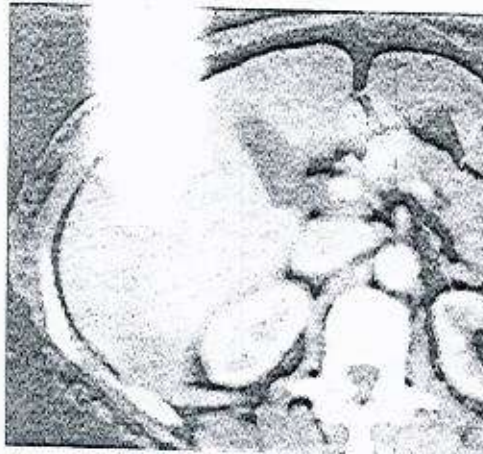


سرعه رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

on the

5 CT-ABDOMEN & PELVIS:

- Enlarged liver showing segment III subcapsular hypodense focal lesion, largest at 2.8 cm along its axial dimensions. No biliary radicles dilatation.
- Distended gall bladder showing large radiodense stone. Average mural thickening.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, kidneys, aorta, IVC and urinary bladder.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- Mesenteric fat smudging with prominent mesenteric lymph nodes.
- No ascites.
- No definite lytic or sclerotic osseous lesions.



OPINION:

**** Reported case of breast cancer showing:**

- Enlarged left breast showing diffuse parenchymal distortion, edema pattern and small soft tissue nodules, for sonomammographic correlation.
- Bilateral axillary, left internal mammary and mediastinal lymphadenopathy as detailed, likely metastatic.
- Scattered bilateral multiple pulmonary nodules, likely metastatic.
- Enlarged liver with segment III and VI focal lesions, possibly metastatic. For further work-up.
- Calcular gall bladder.

Dr. Mahmoud Hanafy, MD

Dr. Ali Elsisy

Patient Name: Kefayah Sayed Mohamed Sallam	Date : 09/05/2024
Exam : MAMMOGRAPHY BOTH	Age : 61 Year(s)
Medical ID: 26208270101281	

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
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Opinion:

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Yours truly ,

DR/Asmaa Zeinhom, Msc